Thyrocare

Tests you can trust

Name : Mohammad Zaid Khan (27Y/M)

Date : 07 May 2024

Test Asked : Rbs, Hcl Tasl Annual Health Check + 3 Others

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NAME : MOHAMMAD ZAID KHAN (27Y/M)

REF. BY : HCL

TEST ASKED : RBS,HCL TASL ANNUAL HEALTH CHECK,HBA,HEMOGRAM - 6 PART (DIFF),COMPLETE URINE ANALYSIS

SAMPLE COLLECTED AT :
, 10, JIGANI INDUSTRIAL AREA, JIGANI, , BENGALURU, KARNATAKA, INDIA,

MOBILE NO :

DOB :

PAN ID :

Summary Report

Tests outside reference range

TEST NAME	OBSERVED VALUE	UNITS	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
PLATELET TO LARGE CELL RATIO(PLCR)	18.8	%	19.7-42.4
RED CELL DISTRIBUTION WIDTH (RDW-CV)	14.2	%	11.6-14
TOTAL RBC	5.73	X 10^6/µL	4.5-5.5
LIPID			
HDL / LDL RATIO	0.27	Ratio	> 0.40
LDL / HDL RATIO	3.7	Ratio	1.5-3.5
LDL CHOLESTEROL - DIRECT	193	mg/dL	< 100
NON-HDL CHOLESTEROL	228.49	mg/dL	< 160
TC/ HDL CHOLESTEROL RATIO	5.4	Ratio	3 - 5
TOTAL CHOLESTEROL	281	mg/dL	< 200
TRIG / HDL RATIO	3.73	Ratio	< 3.12
TRIGLYCERIDES	196	mg/dL	< 150
LIVER			
ALANINE TRANSAMINASE (SGPT)	131.1	U/L	< 45
ASPARTATE AMINOTRANSFERASE (SGOT)	83	U/L	< 35
BILIRUBIN (INDIRECT)	1.07	mg/dL	0-0.9
BILIRUBIN - TOTAL	1.25	mg/dL	0.3-1.2
PROTEIN - TOTAL	8.51	gm/dL	5.7-8.2
SERUM GLOBULIN	3.72	gm/dL	2.5-3.4

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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NAME : MOHAMMAD ZAID KHAN (27Y/M)
REF. BY : HCL
TEST ASKED : BLOOD SUGAR (RANDOM),COMPLETE URINE ANALYSIS,HbA1c,HCL TASL ANNUAL HEALTH

SAMPLE COLLECTED AT :
, 10, JIGANI INDUSTRIAL AREA, JIGANI, , BENGALURU, KARNATAKA, INDIA,

PATIENTID : MK23735280
MOBILE NO :
DOB :
PAN ID :

TEST NAME	TECHNOLOGY	VALUE	UNITS
RANDOM BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	84.6	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Random/Post-Prandial Plasma Glucose (RPG/PPPG)	
Normal	70 to 140 mg/dl
Impaired Glucose Tolerance	140 - 199 mg/dl
Diabetes	Greater than or Equal to 200

Please correlate with clinical conditions.
Method:- GOD-PAP METHOD

Sample Collected on (SCT) : 07 May 2024 08:00
Sample Received on (SRT) : 07 May 2024 18:27
Report Released on (RRT) : 07 May 2024 19:12
Sample Type : FLUORIDE NF
Labcode : 0705098046/HCL72
Barcode : CF651824

Dr Syeda Sumaiya MD(Path)

Dr.Ashwin Mathew MD(Path)

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NAME : MOHAMMAD ZAID KHAN (27Y/M)
REF. BY : HCL
TEST ASKED : BLOOD SUGAR (RANDOM),COMPLETE URINE ANALYSIS,HbA1c,HCL TASL ANNUAL HEALTH
PATIENTID : MK23735280
MOBILE NO :
DOB :

SAMPLE COLLECTED AT :
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PAN ID :

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c - (HPLC)			
	H.P.L.C	5.4	%

Bio. Ref. Interval. :

Bio. Ref. Interval.: As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	108	mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Sample Collected on (SCT) : 07 May 2024 08:00
Sample Received on (SRT) : 07 May 2024 18:36
Report Released on (RRT) : 07 May 2024 19:32
Sample Type : EDTA
Labcode : 0705098486/HCL72
Barcode : CG912755

Dr Syeda Sumaiya MD(Path)

Dr.Ashwin Mathew MD(Path)

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NAME : MOHAMMAD ZAID KHAN (27Y/M)
REF. BY : HCL
TEST ASKED : BLOOD SUGAR (RANDOM),COMPLETE URINE ANALYSIS,HbA1c,HCL TASL ANNUAL HEALTH
PATIENTID : MK23735280
MOBILE NO :
DOB :

SAMPLE COLLECTED AT :
, 10, JIGANI INDUSTRIAL AREA, JIGANI, , BENGALURU, KARNATAKA, INDIA,

PAN ID :

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL LEUCOCYTES COUNT (WBC)	HF & FC	8.24	X 10^3 / µL	4.0 - 10.0
NEUTROPHILS	Flow Cytometry	70.3	%	40-80
LYMPHOCYTE	Flow Cytometry	20.9	%	20-40
MONOCYTES	Flow Cytometry	4.9	%	2-10
EOSINOPHILS	Flow Cytometry	2.9	%	1-6
BASOPHILS	Flow Cytometry	0.7	%	0-2
IMMATURE GRANULOCYTE PERCENTAGE(IG%)	Flow Cytometry	0.3	%	0-0.5
NEUTROPHILS - ABSOLUTE COUNT	Calculated	5.79	X 10^3 / µL	2.0-7.0
LYMPHOCYTES - ABSOLUTE COUNT	Calculated	1.72	X 10^3 / µL	1.0-3.0
MONOCYTES - ABSOLUTE COUNT	Calculated	0.4	X 10^3 / µL	0.2 - 1.0
BASOPHILS - ABSOLUTE COUNT	Calculated	0.06	X 10^3 / µL	0.02 - 0.1
EOSINOPHILS - ABSOLUTE COUNT	Calculated	0.24	X 10^3 / µL	0.02 - 0.5
IMMATURE GRANULOCYTES(IG)	Calculated	0.02	X 10^3 / µL	0-0.3
TOTAL RBC	**HF & EI**	**5.73**	**X 10^6/µL**	**4.5-5.5**
NUCLEATED RED BLOOD CELLS	Calculated	0.01	X 10^3 / µL	0.0-0.5
NUCLEATED RED BLOOD CELLS %	Flow Cytometry	0.01	%	0.0-5.0
HEMOGLOBIN	SLS-Hemoglobin Method	15.8	g/dL	13.0-17.0
HEMATOCRIT(PCV)	CPH Detection	49.3	%	40.0-50.0
MEAN CORPUSCULAR VOLUME(MCV)	Calculated	86	fL	83.0-101.0
MEAN CORPUSCULAR HEMOGLOBIN(MCH)	Calculated	27.6	pq	27.0-32.0
MEAN CORP.HEMO.CONC(MCHC)	Calculated	32	g/dL	31.5-34.5
RED CELL DISTRIBUTION WIDTH - SD(RDW-SD)	Calculated	44.4	fL	39-46
RED CELL DISTRIBUTION WIDTH (RDW-CV)	**Calculated**	**14.2**	**%**	**11.6-14**
PLATELET DISTRIBUTION WIDTH(PDW)	Calculated	9.6	fL	9.6-15.2
MEAN PLATELET VOLUME(MPV)	Calculated	9.2	fL	6.5-12
PLATELET COUNT	HF & EI	323	X 10^3 / µL	150-410
PLATELET TO LARGE CELL RATIO(PLCR)	**Calculated**	**18.8**	**%**	**19.7-42.4**
PLATELETCRIT(PCT)	Calculated	0.3	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

Please Correlate with clinical conditions.
Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)
(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height**)**

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Report Released on (RRT) : 07 May 2024 19:32
Sample Type : EDTA
Labcode : 0705098486/HCL72
Barcode : CG912755

Dr Syeda Sumaiya MD(Path)

Dr.Ashwin Mathew MD(Path)

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NAME : MOHAMMAD ZAID KHAN (27Y/M)
REF. BY : HCL
TEST ASKED : COMPLETE URINE ANALYSIS

SAMPLE COLLECTED AT :
, 10, JIGANI INDUSTRIAL AREA, JIGANI, ,
BENGALURU, KARNATAKA, INDIA,

PATIENTID : MK23735280
MOBILE NO :
DOB :
PAN ID :

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
Physical Examination				
VOLUME	Visual Determination	3	mL	-
COLOUR	Visual Determination	PALE YELLOW	-	Pale Yellow
APPEARANCE	Visual Determination	CLEAR	-	Clear
SPECIFIC GRAVITY	pKa change	1.02	-	1.003-1.030
PH	pH indicator	6.5	-	5-8
Chemical Examination				
URINARY PROTEIN	PEI	ABSENT	mg/dL	Absent
URINARY GLUCOSE	GOD-POD	ABSENT	mg/dL	Absent
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
Microscopic Examination				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	ABSENT	cells/HPF	0-5
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	ABSENT	cells/HPF	0-5
EPITHELIAL CELLS	Microscopy	1	cells/HPF	0-5
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	Microscopy	ABSENT	-	Absent
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase**)**

Sample Collected on (SCT) : 07 May 2024 08:00
Sample Received on (SRT) : 07 May 2024 19:23
Report Released on (RRT) : 07 May 2024 20:14
Sample Type : URINE
Labcode : 0705100325/HCL72
Barcode : CF525502

Dr Syeda Sumaiya MD(Path)

Dr.Ashwin Mathew MD(Path)

NAME	: MOHAMMAD ZAID KHAN (27Y/M)	**SAMPLE COLLECTED AT :**	, 10, JIGANI INDUSTRIAL AREA, JIGANI, , BENGALURU, KARNATAKA, INDIA,
REF. BY	: HCL		
TEST ASKED	: BLOOD SUGAR (RANDOM),COMPLETE URINE ANALYSIS,HbA1c,HCL TASL ANNUAL HEALTH		
PATIENTID	: MK23735280		
MOBILE NO	:	**PAN ID**	:
DOB	:		

TEST NAME	TECHNOLOGY	VALUE	UNITS
HEPATITIS B SURFACE ANTIGEN (HBSAG)	E.C.L.I.A	0.38	OD ratio

Non reactive: < 0.90
Border line : >=0.90 to < 1.0
Reactive : >= 1.0

Clinical Significance:
Hepatitis B Surface Antigen test is a screening test and is being performed using solid phase enzyme immunoassay. A positive report does not confirm diagnosis and all positive cases should be confirmed by confirmatory test like PCR. Type B viral hepatitis is usually accompanied by the appearance of hepatitis B surface antigen in the serum. HBsAg can be detected in the serum as early as 2 to 3 weeks before the onset of the illness and reaches a peak titre at the time when the characteristic symptoms like jaundice and changes in the liver-specific enzymes appear. This is normally followed by a gradual elimination of the antigen. In some cases and in an unknown percentage of subclinical hepatitis b virus infections, the antigen can be detected in the serum for years, if not for life. Despite the high sensitivity of HBsAg assays, a risk of the transmission of hepatitis B by an HBsAg -negative sample cannot be ruled out.

Specifications:
Precision: Intra assay (%CV): 14.6 %, Inter assay (%CV): 11.6 % ,Sensitivity: 99.90%; Specificity:100%

Kit validation references: Proposal for the validation of Anti HIV 1/2 or HIV Ag/Ab combination Assays Anti HCV Assays HBsAg and Anti HBc assay for Use with cadaveric Samples PEI 08/05/2014.

Sample Collected on (SCT)	: 07 May 2024 08:00
Sample Received on (SRT)	: 07 May 2024 19:07
Report Released on (RRT)	: 07 May 2024 22:13
Sample Type	: SERUM
Labcode	: 0705099424/HCL72
Barcode	: CI912610

Dr Syeda Sumaiya MD(Path)

Dr.Ashwin Mathew MD(Path)

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NAME : MOHAMMAD ZAID KHAN (27Y/M)
REF. BY : HCL
TEST ASKED : BLOOD SUGAR (RANDOM),COMPLETE URINE ANALYSIS,HbA1c,HCL TASL ANNUAL HEALTH
PATIENTID : MK23735280
MOBILE NO :
DOB :

SAMPLE COLLECTED AT :
, 10, JIGANI INDUSTRIAL AREA, JIGANI, , BENGALURU, KARNATAKA, INDIA,

PAN ID :

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	**PHOTOMETRY**	**281**	**mg/dL**	**< 200**
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	52	mg/dL	40-60
TRIGLYCERIDES	**PHOTOMETRY**	**196**	**mg/dL**	**< 150**
LDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**193**	**mg/dL**	**< 100**
TC/ HDL CHOLESTEROL RATIO	**CALCULATED**	**5.4**	**Ratio**	**3 - 5**
TRIG / HDL RATIO	**CALCULATED**	**3.73**	**Ratio**	**< 3.12**
LDL / HDL RATIO	**CALCULATED**	**3.7**	**Ratio**	**1.5-3.5**
HDL / LDL RATIO	**CALCULATED**	**0.27**	**Ratio**	**> 0.40**
NON-HDL CHOLESTEROL	**CALCULATED**	**228.49**	**mg/dL**	**< 160**
VLDL CHOLESTEROL	CALCULATED	39.12	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
TRIG - Enzymatic, End Point
LDL - Direct Measure
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

Sample Collected on (SCT) : 07 May 2024 08:00
Sample Received on (SRT) : 07 May 2024 19:07
Report Released on (RRT) : 07 May 2024 22:13
Sample Type : SERUM
Labcode : 0705099424/HCL72
Barcode : CI912610

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NAME : MOHAMMAD ZAID KHAN (27Y/M)
REF. BY : HCL
TEST ASKED : BLOOD SUGAR (RANDOM),COMPLETE URINE ANALYSIS,HbA1c,HCL TASL ANNUAL HEALTH
PATIENTID : MK23735280
MOBILE NO :
DOB :

SAMPLE COLLECTED AT :
, 10, JIGANI INDUSTRIAL AREA, JIGANI, , BENGALURU, KARNATAKA, INDIA,

PAN ID :

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	96.4	U/L	45-129
BILIRUBIN - TOTAL	**PHOTOMETRY**	**1.25**	**mg/dL**	**0.3-1.2**
BILIRUBIN -DIRECT	PHOTOMETRY	0.18	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	**CALCULATED**	**1.07**	**mg/dL**	**0-0.9**
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	25.9	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	**PHOTOMETRY**	**83**	**U/L**	**< 35**
ALANINE TRANSAMINASE (SGPT)	**PHOTOMETRY**	**131.1**	**U/L**	**< 45**
SGOT / SGPT RATIO	CALCULATED	0.63	Ratio	< 2
PROTEIN - TOTAL	**PHOTOMETRY**	**8.51**	**gm/dL**	**5.7-8.2**
ALBUMIN - SERUM	PHOTOMETRY	4.79	gm/dL	3.2-4.8
SERUM GLOBULIN	**CALCULATED**	**3.72**	**gm/dL**	**2.5-3.4**
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.29	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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Report Released on (RRT) : 07 May 2024 22:13
Sample Type : SERUM
Labcode : 0705099424/HCL72
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NAME : MOHAMMAD ZAID KHAN (27Y/M)
REF. BY : HCL
TEST ASKED : BLOOD SUGAR (RANDOM),COMPLETE URINE ANALYSIS,HbA1c,HCL TASL ANNUAL HEALTH
PATIENTID : MK23735280
MOBILE NO :
DOB :

SAMPLE COLLECTED AT :
, 10, JIGANI INDUSTRIAL AREA, JIGANI, , BENGALURU, KARNATAKA, INDIA,

PAN ID :

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
CREATININE - SERUM	PHOTOMETRY	0.86	mg/dL	0.72-1.18
URIC ACID	PHOTOMETRY	7	mg/dL	4.2 - 7.3

Please correlate with clinical conditions.

Method :

SCRE - Creatinine Enzymatic Method
URIC - Uricase / Peroxidase Method

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REF. BY : HCL
TEST ASKED : BLOOD SUGAR (RANDOM),COMPLETE URINE ANALYSIS,HbA1c,HCL TASL ANNUAL HEALTH CHECK HEMOGRAM
PATIENTID : MK23735280
MOBILE NO :
DOB :

SAMPLE COLLECTED AT :
, 10, JIGANI INDUSTRIAL AREA, JIGANI, , BENGALURU, KARNATAKA, INDIA,

PAN ID :

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	136	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	5.99	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	1.13	µIU/mL	0.54-5.30

Comments : SUGGESTING THYRONORMALCY

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3,T4 - Fully Automated Electrochemiluminescence Compititive Immunoassay
USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Sample Collected on (SCT) : 07 May 2024 08:00
Sample Received on (SRT) : 07 May 2024 19:07
Report Released on (RRT) : 07 May 2024 22:13
Sample Type : SERUM
Labcode : 0705099424/HCL72
Barcode : CI912610

Dr Syeda Sumaiya MD(Path)

Dr.Ashwin Mathew MD(Path)

PROCESSED AT :

Thyrocare,
5CA-711, 3rd Floor,
HRBR 2nd Block,
Hennur, Bengaluru-560043

Thyrocare
Tests you can trust

Thyrocare Technologies Limited, D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400703 98706 66333 wellness@thyrocare.com

*9 out of 10 Doctors Trust that Thyrocare Reports are **Accurate & Reliable***

NAME : MOHAMMAD ZAID KHAN (27Y/M)
REF. BY : HCL
TEST ASKED : BLOOD SUGAR (RANDOM),COMPLETE URINE ANALYSIS,HbA1c,HCL TASL ANNUAL HEALTH

SAMPLE COLLECTED AT :
, 10, JIGANI INDUSTRIAL AREA, JIGANI, , BENGALURU, KARNATAKA, INDIA,

PATIENTID : MK23735280
MOBILE NO :
DOB :
PAN ID :

TEST NAME	TECHNOLOGY	VALUE	UNITS
BLOOD UREA	PHOTOMETRY	33.5	mg/dL

Bio. Ref. Interval. :-

ADULT 17 - 43
NEWBORN 8.4 - 25.8
INFANT/CHILD 10.8 - 38.4

CLINICAL SIGNIFICANCE :
Urea is Nitrogenous waste product formed after protein breakdown. Rise in Urea levels leads to Uremia which may cause Kidney failure. Urea levels may be elevated due to renal causes such as acute glomerulonephritis, chronic nephritis, polycystic kidney, tubular necrosis and nephrosclerosis.

Please correlate with clinical conditions.
Method:- KINETIC UV ASSAY.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	119	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.
Method:- CKD-EPI Creatinine Equation

~~ End of report ~~

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CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Neither Thyrocare, nor its employees/representatives assume any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

EXPLANATIONS

- Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range** - Means the range of values in which 95% of the normal population would fall.